

Restoring the Constitutional Lexicon: A New English Translation of Grunya Sukhareva's 1930 Clinical Taxonomy

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Abstract

This document provides a philologically precise English translation of Grunya Efimovna Sukhareva's 1930 paper, *On the Problem of the Structure and Dynamics of Childhood Psychopathies (Schizoid Forms)*. The 1930 text establishes Sukhareva's mature theoretical architecture of the autistic phenotype. It formally distinguishes primary constitutional symptoms from secondary reactive formations, introduces the principle of environmental responsiveness, and documents positive longitudinal adult outcomes. An initial 2022 English translation successfully introduced the document to a broader readership by modernizing its clinical vocabulary. To support rigorous historiographical analysis, the present translation restores the strict structural-analytic lexicon of interwar constitutional psychiatry, preserving the foundational diagnostic boundaries of the era. It functions as the primary-source companion to the manuscript *The Unread Bibliography*, providing direct Anglophone access to the empirical dataset and clinical framework that mid-century Western psychiatry subsequently displaced.

Keywords: Grunya Sukhareva; history of autism; constitutional psychiatry; schizoid psychopathy; Soviet pedology; history of child psychiatry; structural-dynamic method; primary source translation.

Introduction

I. The Companion Function and the Document's Position

This translation of G. E. Sukhareva's 1930 paper *K probleme struktury i dinamiki detskikh psikhopatii (shizoidnye formy)* [On the problem of the structure and dynamics of childhood psychopathies (schizoid forms)] operates as the companion primary-source document to *The Unread Bibliography: Sukhareva's Taxonomic Architecture and the Eighty-Year Structural Displacement in Autism Research* (Brown, forthcoming). The main paper documents the archival, institutional, and textual mechanisms through which the constitutional-typological framework developed in this 1930 paper was structurally displaced from the canonical autism literature over eight decades. The translation exists to make the primary document accessible in English with the philological exactitude the main paper's forensic thesis requires. Readers approaching the translation independently are directed to the main paper for full historiographical development. Readers approaching the main paper are directed here for direct engagement with the source text whose displacement the forensic argument documents.

Grunya Efimovna Sukhareva (1891–1981) trained in the pre-Revolutionary tradition of Russian neuropathology and spent her clinical career at the intersection of Soviet institutional pedology, constitutional psychiatry, and the study of the *trudnye deti*, the behaviorally difficult children who populated the specialized children's homes and psycho-neurological clinics of the 1920s Soviet state. The clinical material analyzed in the present paper was assembled at the Children's Psycho-Neurological Clinic of the People's Commissariat of Health and at the children's home for behaviorally difficult girls over the period 1923–1930. The paper was delivered at the First All-Union Congress on Human Behavior in Leningrad in January 1930 and published in the *Zhurnal Nevropatologii i Psikhiiatrii imeni S. S. Korsakova* the same year. The institutional framework within which this methodology was developed and practiced, Soviet pedology, was suppressed by decree of the Central Committee of the Communist Party of the Soviet Union in July 1936. The *postanovlenie* of that year condemned pedology as a pseudo-science, banned pedological practice in Soviet schools, and mandated the destruction of pedological literature. Sukhareva survived the purges and continued a distinguished career in Soviet child psychiatry until 1981. Her post-1936 publications were forced to conform to a revised, state-mandated disciplinary architecture. This political suppression of pedological science necessitated the strategic, dual-register writing methodology she subsequently deployed to preserve her constitutional findings.¹

The 1930 paper occupies a specific position within Sukhareva's larger corpus. By 1930, she had produced the 1925 Russian monograph and its 1926 German derivative on the male schizoid clinical phenotype, the 1927 Russian and German papers on the female clinical presentation, and a systematic German-language differential diagnostic program of seven further publications mapping the schizoid profile against the cycloid and manic-depressive types, intellectual disability, toxic-organic neurasthenia, and the infantile gracile body type.² The 1930 paper explicitly positions itself against this background as a methodological deepening: where the

¹On Sukhareva's biographical and institutional trajectory, see Sher, D. A. and Gibson, J. L., *Pioneering, prodigious and prescient: Grunya Efimovna Sukhareva's life and contribution to conceptualising autism and schizophrenia*, *European Child & Adolescent Psychiatry* 32, no. 3 (2023): 475–490. On the 1936 pedology suppression and its consequences for the discipline within which the 1930 paper was produced, see Brown (forthcoming), Section 2.3, and the Vygotsky reference at the same location.

² The full pre-1930 German-language differential diagnostic program is documented in Brown (forthcoming), Appendix A. The primary publications are Ssucharewa & Ossipowa (1926a, 1926b), Ssucharewa (1927), Ssucharewa (1928a, 1928b, 1928c), Ssucharewa & Ossipowa (1929), and Ssucharewa & Schenfil (1929).

prior works isolated individual nosological forms, the 1930 paper analyses the dynamics of their development across successive stages of childhood in dependence upon the interaction between the child's biological constitution and the surrounding social, domestic, and environmental constellation. The empirical basis is a follow-up study of thirty-one cases, eighteen boys and thirteen girls, aged eight to nineteen, observed over five to seven years.

The paper carries specific evidentiary weight within the main paper's forensic thesis. The primary-secondary symptom stratification developed here (Sections 2.3 and 3.3 of the main paper), the positive catamnestic outcomes documented among the nineteen cases who had passed through puberty (Sections 3.3, 7.5, and 8 of the main paper), and the environmental-responsiveness principle Sukhareva formalizes in the paper's fundamental structural claim (Sections 3.3 and 7.7 of the main paper) constitute the core clinical architecture whose absence from the DSM-5-TR and ICD-11 the main paper documents. The 1930 paper is the primary document on which the four criteria-revision targets specified in the main paper's Conclusion empirically rest. The translation exists to make that primary document accessible with the philological precision that argument requires.

The paper's routing to the domestic Russian literature reflects a strategic international communication choice. It did not result from bibliographic oversight. Sukhareva had introduced the schizoid clinical phenotype to the German literature in 1926 and 1927 and directed her subsequent German-language submissions in 1932 and 1933 toward the active Continental debate regarding the course and prognosis of progressive childhood schizophrenia. The 1930 structural-dynamic synthesis of the stationary phenotype consequently remained within the Russian-language literature. Its thirty-one-case longitudinal dataset, its structural-dynamic analysis, and its documentation of positive adult outcomes remained accessible only to readers of Russian. The core constitutional-typological framework nonetheless reached the English-speaking academic canon through separate transmission channels. Eugen Kahn formally integrated the diagnostic boundaries from her German derivative papers into his 1931 Yale University Press textbook, *Psychopathic Personalities*. This publication firmly embedded Sukhareva's stationary neurotype within the foundational curriculum of American psychiatry.

The subsequent displacement of this architecture operated through a process of selective nosological engagement. Leo Kanner and Hans Asperger structurally omitted this stationary constitutional literature when publishing their foundational autism papers in 1943 and 1944. They cited Sukhareva's progressive childhood schizophrenia research exclusively, utilizing the severe disease process as an empirical contrast to claim their own stationary autistic cohorts were unprecedented. This targeted omission collapsed the precise diagnostic boundary Sukhareva had secured. Mid-century Western psychiatry consequently subsumed autism under the broad umbrella of childhood schizophrenia. The field operated within this conceptual vacancy, losing the primary-secondary symptom distinction and the environmental-responsiveness principle, until the formal separation of the diagnoses upon the publication of the DSM-III in 1980.

II. The Theoretical Lexicon and Its Historical Specificity

The 1930 paper operates within the structural-analytic current of continental psychiatry, a tradition associated in the German-language literature with the constitutional psychopathology of Karl Birnbaum, the multi-dimensional diagnostic framework of Ernst Kretschmer, and the primary/secondary symptom differentiation of Eugen Bleuler.³ Sukhareva declares this

³ On the structural-analytic tradition in continental psychiatry and its Soviet pedagogical reception, see Kretschmer, *Körperbau und Charakter*, 7th and 8th rev. ed. (Berlin: Julius Springer, 1929), with Sukhareva and Lazar identified in the Introduction as the two authors extending the constitutional framework into childhood.

theoretical inheritance in the paper's opening sentences and organizes her clinical exposition around four technical constructs whose exact translation is inseparable from the argument's coherence: the architectonics of the personality (Birnbaum), the psychaesthetic proportion of mood (Kretschmer), the primary/secondary symptom distinction (Bleuler), and the sthenic/asthenic constitutional axis (Kretschmer).⁴ Any translation that does not reproduce these constructs with disciplinary exactitude dismantles the theoretical framework within which Sukhareva's findings were produced and from which they derive their clinical meaning.

The categorical distinction between psychopathy and psychosis governs every terminological decision in the translation and warrants explicit statement here. In 1930s constitutional psychiatry, and specifically within the Kretschmerian tradition Sukhareva deploys, *психопатия* (*psikhopatiiā*, psychopathy) and *психоз* (*psikhoz*, psychosis) designate categorically distinct clinical phenomena. Psychopathy denotes a stable constitutional deviation of character. It constitutes a deviation of type, entirely distinct from a progressive disease process, and remains fully compatible with a preserved reality-orientation and with a satisfactory social prognosis. Psychosis denotes a process-based, progressive pathological state, the paradigm case being schizophrenia, which carries an entirely different prognostic and nosological weight. Sukhareva's paper is, at its argumentative core, a demonstration that schizoid psychopathy can be reliably distinguished from incipient schizophrenia, and that the satisfactory prognosis of the former, documented through the catamnestic data, confirms the validity of its nosological delimitation from the latter. The translation renders *психопатия* as psychopathy and *психоз* as psychosis throughout, without approximation in either direction, because the paper's central argument cannot be reconstructed in English if the two categories have been merged in the reader's encounter with the text.

The Kretschmerian and Bleulerian constructs are governed by the same requirement. Psychaesthetic proportion designates a specific quantitative ratio between the poles of hypersensitivity and affective coldness within the schizoid temperament: a structural term, not an evaluative one, whose precision is inseparable from Kretschmer's constitutional psychology. Sthenic and asthenic designate the fundamental constitutional axis of active, energetically vital orientation versus passive, withdrawing orientation. Autism, as Sukhareva deploys the term, is a Bleulerian symptomatic descriptor: the withdrawal of affective investment from the external world into an interior one. Sukhareva deployed it strictly as a feature of the schizoid clinical picture. It did not function as a standalone diagnostic category. The retrospective identification of continuities between Sukhareva's descriptions and the modern autism diagnostic category is a task for the main paper. A translation that performs that identification within the primary text substitutes the reader's interpretive frame for the document's historical one.

III. The New & Kyuchukov Translation and the Philological Requirement

In February 2022, Irina M. New and Hristo Kyuchukov published an English translation of this paper in *European Child & Adolescent Psychiatry*, making a primary document of Sukhareva's mature clinical thought available in English for the first time. That translation successfully introduced the document to a broader readership by modernizing its clinical vocabulary for accessibility and historical introduction. The present translation is a specialized clinical restoration addressed to historians of medicine and psychiatry, psychiatrists and psychologists working within the constitutional and structural-analytic traditions, and scholars of the history of autism who require the specific theoretical lexicon of the original to be reproduced with philological exactitude.

⁴ The full contextualization of this tradition within the pre-1943 European clinical canon and its transmission to Anglophone psychiatry is developed in forthcoming publication.

The philological warrant for undertaking a second English translation rests on the historiographical costs of this modernization. In adapting the text for a contemporary audience, the 2022 translation introduces three terminological shifts that alter the document's historical epistemology. It also features one significant omission.

The first shift occurs at the paper's threshold. The opening sentence renders Sukhareva's «Сложность проблемы конституциональных психопатий» as "The complexity of the constitutional psychoses." Translating this term through a modern lens collapses a fundamental historical category. It overwrites the specific domain of Sukhareva's inquiry at the precise moment she announces it. Sukhareva's paper is a sustained demonstration that schizoid psychopathy can be reliably distinguished from schizophrenic psychosis, and that the satisfactory prognosis of the former confirms the validity of its nosological delimitation from the latter. The 2022 translation renders the term correctly in subsequent passages, but the inconsistency at the article's threshold is highly consequential, because the reader arrives at the argument having already been told it concerns a different clinical category.

The second shift affects the Kretschmerian construct. Психэстетическая пропорция, the Russian rendering of Kretschmer's *psychästhetische Proportion*, designates the specific structural ratio between hypersensitivity and affective coldness within the schizoid temperament. The 2022 translation renders this as "mood imbalance." This modernization introduces a normative valuation absent from the original construct, replacing a technical structural term with a generic clinical descriptor. It inadvertently severs the text from the specific intellectual tradition within which it was produced and forecloses the reader's access to the Kretschmerian constitutional psychology that provides the paper's theoretical architecture.

The third shift affects the Bleulerian term *autizm*. The 2022 translation permits the term to carry connotations of the contemporary diagnostic category. Sukhareva deploys *autizm* strictly in its Bleulerian symptomatic sense: the withdrawal of affective investment from the external world into an interior one. Sukhareva deployed it strictly as a clinical feature of the schizoid picture. It did not function as a standalone diagnostic identity. Amplifying the term toward the contemporary sense assimilates Sukhareva's 1930 clinical description to the framework that displaced her tradition, executing the precise assimilation the main paper's forensic thesis documents.

The 2022 translation also omits a methodologically important subordinate clause specifying the grounds on which congenital psychopathies are excluded from the study. The clause is essential to the paper's methodological transparency and to the reader's understanding of the case-selection criteria that produced the thirty-one-case cohort. The omission does not affect the general readability of the 2022 translation for a non-specialist reader. It remains material to the scholarly and archival functions that a translation of a primary historical document must serve.

These translational decisions share a structural feature: each assimilates Sukhareva's 1930 vocabulary toward the contemporary clinical lexicon in a direction that dissolves the specificity of her constitutional-psychiatric framework. The philological warrant for the present translation is that a document whose central argument concerns the categorical separation of psychopathy from psychosis cannot serve its evidentiary function within the main paper's forensic thesis if the reader encounters that argument through a translation that has already merged the two categories.

IV. Translation Principles, Typography, and Terminological Apparatus

To assist the Anglophone reader while maintaining philological fidelity, key diagnostic terminology appears upon first use in tripartite form: original Cyrillic, ALA-LC transliteration, and English rendering (e.g., психопатия [*psikhopatiiā*], psychopathy). The ALA-LC system is

applied throughout. Soft (ь) and hard (ъ) signs are suppressed for readability except where philologically significant.

Typographical fidelity to the 1930 publication is maintained to preserve functional argument structures. *Разрядка* (spaced lettering) marks the Conclusions heading. Case-study material is set in reduced type. Untranslated German clinical terms (e.g., *Katatonische Bewegungsdrang*) are preserved within the English prose to accurately reflect the intensive German-language orientation of interwar Soviet psychiatry.

The translated text utilizes three distinct marker systems. Arabic numerals (1 2) designate Sukhareva's original authorial footnotes, collected immediately following the translation. Lowercase letters (a b c) designate the translator's terminological endnotes, providing disciplinary-historical context for specific theoretical constructs. Bracketed numerals set in lighter, reduced-size text ([p. n.]) indicate original page breaks, facilitating parallel text comparison with the Russian source document.

1930

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**ON THE PROBLEM OF THE STRUCTURE AND DYNAMICS OF
CHILDHOOD CONSTITUTIONAL PSYCHOPATHIES (SCHIZOID FORMS)¹**

G. E. Sukhareva

[p. 64] The structural-analytical approach occupies a prominent place among the currents of contemporary psychiatry. The problem of the construction of the psychosis (Birnbaum), of multidimensional diagnosis (Kretschmer), of the differentiation of primary and secondary symptoms (Bleuler) — all of these questions represent a definite stage in the development of psychiatric thought. This new direction in psychiatry has also exerted its influence in the domain of the psychopathies. A number of authors, proceeding from the most varied standpoints (the somato-biological [Ewald], the psychopathological [Kahn], the hereditary-biological [Hoffmann]), have raised the question of the necessity of replacing the clinical-descriptive method of studying the psychopathies with the structural-analytical method, the ultimate goal of which is the analysis of the deep connections and interrelations among the individual components of the psyche (the construction of the *architectonics of the personality*, in Birnbaum's sense).

The complexity of the problem of the constitutional psychopathies makes it necessary to approach its resolution from various sides and by different routes. The present communication represents a methodological attempt to approach the study of the psychopathies by a path different from that taken in our earlier works on childhood psychopathies. Whereas in a series of works based on child material we set ourselves the task of isolating individual nosologically more or less delineated forms of childhood psychopathies, of describing the somatopsychic syndromes that characterize them, and thereby of approaching the question of their biological substrate — in the present work we face a different task: to analyse, on the basis of our clinical material, the *dynamics* of individual forms of constitutional childhood psychopathies, and to trace [p. 65] how the psychopathological picture unfolds in dependence upon the child's development and upon the social, domestic, and life-situational factors surrounding him.

When we approached the study of the psychopathies by means of the dynamic method, a further dimension of this problem was opened to us: the question of the *structure* of individual forms of psychopathy. Our attention was drawn, in the first instance, to the complexity of this structure. In analysing the development of the psychopathological picture in each individual case of

psychopathy, we could not fail to note the unequal significance of the various symptoms from the standpoint of their pathogenesis and the mechanism of their emergence. Alongside those symptoms that are specific and primary to the given form of psychopathy, constituting the immediate expression of the biological insufficiency, we noted a further series of supplementary symptoms: *secondary reactive formations*, which must be regarded as an adaptation of the remaining personality to the given insufficiency, representing the product of various compensatory mechanisms that adapt the personality, with its specific deficiency, to the given life situation. Only the general direction in the development of these compensatory mechanisms, the path along which the reaction proceeds, depends upon the personal predisposition of the given subject; the content and form of the reaction are, to a considerable degree, determined by external factors: the environment, upbringing, the level of development, the given life situation, and so forth.

From this constant combination of the endogenous and the exogenous, of *constitution* and *constellation*, from the combination of the primary basic symptoms with secondary reactive accretions, there results a highly complex structure of the psychopathological picture in each individual case. The separation of the primary basic symptoms from the secondary supplementary and reactive formations is the fundamental task of all structural analysis.

In order to approach the resolution of the questions we have posed, we undertook the collection of follow-up information on those cases of childhood psychopathy that had been observed by us at the Children's Psycho-Neurological Clinic of the People's Commissariat of Health and at the children's home for behaviourally difficult girls over the preceding seven years.

The tasks we had set ourselves required careful selection of the material admitted to analysis. We confined ourselves, in the first instance, to the group of constitutional psychopathies, excluding all congenital psychopathies (in which one can speak only of damage to the germinal endowment, without constitutional factors being present) and all exogenously determined psychopathic states. From among the individual cases of constitutional psychopathy, we selected the more clearly defined, more pronounced, and most extensively followed cases (observed for no less than five years). For the convenience of exposition, [p. 66] we divided the entire material into groups according to constitutional type: schizoids, cycloids, epileptoids, and so on. In the present communication we present our material for the schizoid psychopathy group. It must be noted that we employed the term *schizoid* not in the Kretschmerian sense, not in its broad meaning; we assigned to this group the more severe, more circumscribed forms of psychopathy, with a definite psychopathic substrate. In total, we followed 31 cases — 18 boys and 13 girls from various social backgrounds; age ranged from 8 to 19 years, duration of observation from 5 to 7 years; of our cases, 19 individuals had already passed through puberty. We proceed to the exposition of our findings.

In studying the dynamics of any form of psychopathy, it is of considerable importance to establish how early the first symptoms make their appearance.

On the basis of our material we were unable to obtain an exhaustive answer to this question, since the children came to us only at school age. Information about early childhood and the preschool period could be drawn only from anamnestic data, which in this regard are not sufficiently complete. Nevertheless, in 6 cases we obtained information that already at the age of 2–3 years the child appeared unlike other children; most frequently the parents identified the first symptoms of the schizoid's insufficiency during the preschool period (14 cases), and in only 2 cases were the first symptoms noticed at school age (in 9 cases — those of children from children's homes — no information was available).

Inseparably connected with this is the question of which symptoms belong to the earliest and of how the subsequent unfolding of the clinical picture proceeds. In our material, the earliest

symptoms fall within the domain of psychomotor disturbances. This is entirely intelligible, since at an early age, when the psyche is as yet little differentiated, the majority of parental complaints concern precisely this sphere: complaints of heightened motor excitability or of sluggishness, of instability. The psychomotor insufficiency of schizoids, however, differs sharply from what is encountered in other preschool children. At a very early stage there is already manifest the characteristic schizoid bipolarity between excitation and sluggishness; and the excitation of the schizoid is distinguished by its stereotypy and purposelessness — it is not directed toward play or toward any occupation, and in its pathological intensification resembles the *Katatonische Bewegungsdrang*.^e

The slowness and sluggishness of the schizoid is reminiscent of catatonic rigidity: these children eat slowly, chew their food for prolonged periods, dress slowly, begin to do something and then suddenly stop. Likewise, the quality of movements — their clumsiness and angularity — early draws the attention of parents. Among the early symptoms must also be counted the bipolarity between automatic [p. 67] compliance and negativism, characteristic of the catatonoid forms: soft, obedient, and indecisive, given to imitating all the other children, these individuals frequently and unexpectedly display sharp, irresistible negativism. Somewhat later, the features characteristic of schizoids in the sphere of instinctual life and emotional reactions make their appearance. Their characteristically weak need for emotional ties, their weak attachment to those around them, are frequently evident already in early childhood; these are often unaffectionate children, with a certain emotional coolness about them.

Less frequently, though nonetheless noted even in the preschool period, are elements of the *psychaesthetic proportion* of mood: a slight vulnerability in a specific direction, combined with general emotional sluggishness, and a weak capacity for the abreaction of emotional experiences.^a The emotional features are particularly early in their manifestation in schizoid girls: the inadequacy of emotional reactions, the bizarreness of emotional combinations, marked ambivalence, and so on. The intellectual features of schizoids (a tendency toward the abstract and the formal, bizarreness and automatism of associations) ordinarily manifest later than the psychomotor disturbances, but already in the preschool period the marked distractedness and self-absorption of these children, as well as the peculiar orientation of their interests, frequently attract attention. They do not join other children in communal play; they devise games for themselves alone; they take no interest in the activities of the nursery school. The tendency to ask questions, characteristic of this developmental phase, frequently takes on an obsessive quality in schizoid children; and their questions are often of an abstract character, not infrequently coloured by hypochondriacal elements, a fear of death, and a dread of life.

I probably won't live very long, one of our young patients used to say; another was afraid of growing older: at 5, he was afraid of turning 6; at 6, afraid of turning 7.

The school period — the beginning of formal instruction and life within a collective — constitutes a powerful stimulus for schizoids and produces a further unfolding of the clinical picture with all its supplementary accretions. The peculiarities of the schizoid's social orientation now begin to emerge distinctly. In preschool children we do not yet find markedly expressed autism; while there may be elements of unsociability and withdrawal, it is not yet appropriate to speak of genuine seclusion and secretiveness. The development of these traits must be assessed as a further shift in the schizoid personality — as a reaction to inadequate adaptation to the surrounding social environment. To the weak contact with the surrounding world, which is a characteristic feature of schizoids, there is now added the further factor of traumatization from contact with the children's collective. The marked motor backwardness and the eccentricity [p. 68] of the schizoid render him incomprehensible to the rest of the child group. He provokes a series of mockeries and jibes from his peers, and the reaction to this is a yet deeper self-enclosure, a withdrawal from the collective.

I was not like the other children — the other boys are agile at games, they won't have me — the character of children is such that they choose the strong, thus a boy of 12 recounts of himself. Another recounts: I was always clumsy, I couldn't play with the children, I only played such games as could be played without companions.

The next symptom that begins to manifest with particular prominence at school is hebephrenic behaviour: mannerism, grimacing, a tendency toward silliness and absurd games. This feature is often present in preschool children as well, but becomes especially pronounced toward the onset of the prepubertal period. The appearance of these features at school may be regarded not only as a certain developmental stage in the manifestation of the schizoid components, but also as a specific adaptive reaction to the school environment; the child attempts to occupy some position in the collective in which he is not recognized, and the most readily available position is chosen: that of the class fool.

During the school period there appear a number of new syndromes that represent the product of the constant interaction of the basic constitutional features with the specific environment and with life experience. Of these, we shall dwell only upon two that were encountered most frequently in our material: the *psychasthenic* and the *paranoid*. The psychasthenic syndrome is most frequently encountered as a reaction to poor adaptability to the demands of the surrounding school environment in a particular type of schizoid: the soft, compliant type, with reduced vital tone. Isolation from the collective, combined with unrelenting mockery, generates a sense of inadequacy, fear of the stronger peers, and thence timidity, indecisiveness, and an asthenic orientation toward those around them. Evidence of the reactive character of this syndrome is provided by those cases in our material where it is inconstant and disappears upon a change of situation. The reactive character is still more clearly apparent in the formation of the paranoid syndrome. The most favourable soil for its emergence is created when a schizoid who is emotionally cold, sthenic,^b with heightened self-regard and egocentrism, finds himself in conditions that wound his self-esteem. The constant desire to distinguish himself and to surpass others, combined with an awareness of his own inadequacy, generates an anxious emotional background, suspiciousness, and a distrustful attitude toward those around him. The rigidity and inflexibility of the schizoid contribute to the emergence of a characteristic feature of the paranoid symptom-complex in the schizoid — an exaggerated sense of justice, a constant desire to seek out the truth in all circumstances, a particularly and unreservedly demanding attitude toward others combined with insufficient [p. 69] demands upon himself. This paranoid syndrome is little noticeable at an earlier age and becomes more prominent in parallel with the growth of the personality.

Thus we observe that by the onset of puberty the schizoid personality has already taken shape in its main features; in certain cases we were able to note even a smoothing-over of some of the pathological accretions, as though the personality had adapted itself to the surrounding conditions. The onset of puberty is marked by new dynamic shifts. The clinical picture of these pubertal changes is highly varied. There are, in the first place, a number of features that are physiologically characteristic of this period and are not connected with any specific constitutional type: heightened excitability, instability, egocentrism, coarseness, a reduction in intellectual productivity, and changes in the motor sphere and in bodily constitution.

Without dwelling upon these, we shall note only what is specific to the schizoid predisposition as it enters into the picture of pubertal changes.

The timing of the onset of puberty in schizoids shows more variation than in other types. Of 13 girls, 5 cases presented delayed onset of menstruation, at 15–16 years; 3 cases showed unduly early onset, at 11 years. The primary symptom of the pubertal period — the predominance of sexual interests over all others — assumes a somewhat distinctive character in schizoids. In their

sexual strivings they are less directed toward the opposite sex. The most frequent expression of heightened sexuality in schizoids (particularly in boys) was masturbation, which frequently assumes a persistent character. Homosexual tendencies, which are in any case characteristic of this age, occur considerably more frequently in schizoids and in certain cases assume the character of pronounced perversions (we noted these in 4 cases). In those cases where interest in the opposite sex was present, a series of features specific to the schizoid was likewise observed: marked ambivalence toward the object of attachment combined with outward coarseness, and, alongside this, considerable tenderness and sentimentality.

The course of the pubertal period in schizoids is not only distinctive in its clinical picture but also more severe. Here, pubertal changes more frequently attain a degree of severity at which it becomes possible to speak of *psychopathic crises*. There were 4 such cases in our material.

In their clinical picture, these crises represent a deterioration of the general state and an intensification of the basic schizoid features. The duration of these pubertal deteriorations ranges from several months to one year; the prognosis is good; in all our cases we [p. 70] observed a restoration of the prior state. Judging from our follow-up data, after the pubertal period there follows a smoothing-over of many of the psychopathic features; the adolescents become more even-tempered and composed. Two cases of schizophrenia (to be discussed below) constitute an exception.

In other words, we may state that the follow-up material we have assembled gives us the right to assign a satisfactory prognosis to our schizoid psychopathy cases, both in respect of the influence upon them of the therapeutic-pedagogical institution and in respect of their subsequent fate. Of particular importance for the assessment of the prognosis in the schizoid group is the question of the possibility of subsequent onset of schizophrenia. Of all our cases, we had the development of a schizophrenic process during puberty in only two; in all the remaining cases the follow-up is entirely satisfactory: the curve of their development shows no tendency toward deterioration; on the contrary, as noted above, a definite equalization of the pathological aspects of the psyche is observed upon the conclusion of the pubertal period. Thus, of 19 individuals who had passed through puberty, 6 had found employment in production and were living independently; 9 had completed the seven-year school. Some of these were continuing their studies at specialized courses or at institutions of higher learning. (It is true that the adaptation to life of these adolescents frequently encounters difficulties: they show helplessness and bewilderment in many of life's questions; they are low in initiative and require external support. But it must be taken into account that the most pronounced forms of schizoid psychopathy were selected for the present work.) The satisfactory prognosis of this group of psychopathies confirms the correctness of its delimitation from the mild forms of schizophrenia.

It must be said, however, that among the severe forms of schizoid psychopathy, those with low productivity and a large number of neurotic superstructures, there exist cases that, given the present state of our knowledge, cannot be delimited from mild, slowly progressing schizophrenias. With respect to these severe forms (in the sense of their delimitation from schizophrenia), we consider it necessary to express ourselves more cautiously than in our first paper on schizoid psychopathy.

From all that has been set forth above concerning the dynamics of schizoid psychopathies, it follows that the basic features of this group appear early and, as a definite developmental tendency, pass through all the developmental stages; but at different periods of childhood now one, now another feature comes to the foreground. Those symptoms that are closest to the biological roots, that constitute the primary basis of the morbid predisposition, belong among the earliest. The older the child, the more differentiated his [p. 71] psyche, the more complex and rich is the

symptomatology of his schizoid psychopathy, and the greater the place occupied by secondary, reactive, and compensatory formations in the clinical picture.

By means of the analysis of the dynamics of schizoid psychopathies, then, we found it easier to isolate, from the complex psychopathological picture, the *primary basic psychic symptoms*. These reduce to the following:

1. disturbances of psychomotor function — the ineptitude and angularity of movements, the characteristic bipolarity between excitation and sluggishness, automatism, and stereotypy;
2. disturbances of instinctual life and emotional reactions, expressed in weak affective attachment to those around them and in the absence of unity in emotional experience;
3. features of associative activity and thinking: bizarreness of associations, a tendency toward the abstract, automatism, and insufficient flexibility.

These basic symptoms are the immediate psychic expression of the schizoid's biological insufficiency. Where are the roots of this insufficiency to be sought? In the literature on this question, as well as in our own previous works on schizoid psychopathies, there is advanced the hypothesis that the biological substrate of this group is to be sought in a congenital insufficiency of specific cerebral systems (the extrapyramidal system) and in some anomaly of the endocrine apparatus.

Our subsequent observations have confirmed this hypothesis and give us reason to believe that this biological insufficiency can account for the basic features of the schizoid psyche. The ineptitude and angularity, the poor adaptability in all motor acts, extends also to a series of other functions in the intellectual and emotional life of schizoids. The schizoid is awkward and inflexible in his entire social orientation. His instinctual life is constituted in a peculiar fashion, as a result of which his contact with the external world is frequently bizarrely strange and always deficient.

Such are the basic symptoms of schizoid psychopathies. The description of the secondary symptoms and of the supplementary reactive formations must be the subject of the next part of the present work. The scope of this article has not permitted us to address this problem in its entirety. For this reason, in the clinical analysis we fixed our attention only upon the following point: is there any connection between the reaction and the constitutional background upon which it arises? Can any regularity^f be discerned within this diversity of complex characterological constructions? On the basis of our data we must answer this question in the affirmative. In the first place, in studying our material we could not fail to notice the fact that the schizoid psyche is most vulnerable [p. 72] with respect to specific stimuli. These include all those factors that are connected with the wounding of the patient's personality — his ego² — and give rise to conflictual experiences: harsh upbringing with constant coercion and humiliation, severe life situations that wound self-esteem and provoke a sense of personal inadequacy. Not infrequently we also noted, in the personal histories of our patients, the traumatizing role of various family disturbances: conflict between the parents, disputes over the child, forced separation from one of them, and jealousy toward one or another member of the family. (It is of interest to note that such factors as material insufficiency, inadequate supervision, and exposure to the street environment play a considerably smaller role here, whereas in the group of cycloid psychopathies it is precisely these factors that play the primary role in the development of reactive states.)

It is important to note, moreover, that the schizoid psyche responds to these external noxious factors with *specific* reactions.

The most frequent form of reaction in our material consisted of various *neurotic states* (observed in one third of all our cases). It may be supposed that the frequency of this reaction in schizoids is

not accidental, since the schizoid psyche (in its characteristics of instinctual life, in the absence of unity in experience, and in the weak abreaction of emotional experience) provides a series of prepared mechanisms for their emergence. In the interest of abbreviating the present work, we omit the description of all the forms of neurotic states observed in our material; their clinical picture was highly varied, ranging from mild forms, in which the matter reduced itself to heightened irritability, sleep disturbances, and nocturnal fears, to more severe forms, in which the symptoms of obsessive states and hysterical reactions were already present. We likewise omit the analysis of the structure of these neurotic constructions in the schizoid, considering it necessary to emphasize only that they frequently assume a prolonged course here, show a tendency toward fixation, and respond poorly to therapeutic intervention.

Another frequent form of reaction in schizoids to external noxious factors consists of various *characterological shifts* — prolonged psychogenic changes of character. In studying these characterological shifts, the following fact drew our attention: there is no sharp boundary here between the psychopathic reaction and the psychopathic state, for the schizoid psyche responds to the most powerful stimuli with reactions that are in many cases already irreversible, as a result of which the entire [p. 73] structural plan of the personality is altered and a picture is produced that, in contrast to acute reversible states, may be characterized as *pathological personality development*. In our cases this pathological personality development proceeded along two lines: first, along the line of intensification of the basic features of the schizoid psyche (in unfavourable circumstances the schizoid becomes more sluggish and more distractible, more withdrawn and more autistic, and so forth); and second, along the line of the appearance of new symptoms, not specific to any constitutional type.

Such symptoms include coarseness, embittered hostility, cruelty, and distrustfulness. Schizoid psychopaths who have lived under severe conditions display, in their affective manifestations, a shift in the direction of the anaesthetic pole — they are for the most part cold, coarse, and obstinate subjects. There is no difficulty in accounting for the mechanism of these character changes: the severe external conditions turn the schizoid against the surrounding environment; he becomes distrustful and embittered; unable to assert his ego by appropriate means, he frequently acts in a paralogical fashion,^c exaggerating all his negative traits, deliberately being coarse and insolent.

Schizoid psychopaths who have spent several years in poorly organized children's homes present a characteristic picture of such pathological personality development. They are more insolent, more coarse, more negativistic. The general emotional tone is more sullen; the paranoid syndrome occurs in them more frequently than in other schizoids.

In a number of further examples from our material we were able to note various characterological shifts: now in the direction of emotional blunting, now in the direction of heightened irritability. In many cases of complex characterological construction, it is not easy to determine the origin of a given syndrome: whether it has been produced by an external factor or has merely been provoked by a specific experience in a correspondingly predisposed subject. Most frequently, the resolution of this question may be found in both the specific features of the given personality and of the given experience.

From the structural analysis of schizoid psychopathy presented above, it may be concluded that external factors are capable of modifying the development of the schizoid character, that the clinical picture of schizoid psychopathy does not represent something fixed, and that over the course of the subject's life, under the influence of environment and experience, it undergoes a series of oscillations in one direction or another. The fundamental path of these oscillations — the direction of the reactions — remains within the bounds of the given constitution.

CONCLUSIONS^d

1. The development of schizoid psychopathies must not be regarded as the passive unfolding of a genetically determined insufficiency; it is a dynamic phenomenon, a process of adaptation of the given personality to the given environment.
2. Schizoid psychopathy manifests its basic symptoms already in early childhood. The development of its symptomatology frequently proceeds unevenly, producing numerous jolts and shifts under the influence of exogenous factors.
3. In the picture of schizoid psychopathies, two series of symptoms must be distinguished: (a) the *primary basic symptoms*, which are the immediate psychic manifestations of the biological insufficiency; and (b) the *secondary symptoms*, in the form of various reactive states and characterological shifts, representing the product of the complex interaction of endogenous and exogenous factors.

AUTHOR'S NOTES

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²A separate work is to be devoted to this question.

TRANSLATOR'S NOTES

^a *Отреагирование* (*otreagirovanie*): the Russian rendering of the psychoanalytic concept *Abreagieren* — the cathartic discharge of emotionally charged experience through its verbal or active expression. Sukhareva's use of this term in a 1930 text reflects the active circulation of psychoanalytic concepts within Soviet psychiatry of the late 1920s. Rendered throughout as *abreaction*, preserving the specific theoretical resonance of the original.

^b *Стеничный* (*stenichnyi*): in the constitutional psychiatry of this period — specifically in the Kretschmerian framework and its Soviet reception — *sthenic* (from Greek *sthenos*, strength) designates an active, energetically vital, assertive personality type, as opposed to the *asthenic* type, characterized by passivity, weakness, and withdrawal. The opposition sthenic/asthenic is a fundamental axis of constitutional characterology in this tradition. The term is not to be confused with its physiological sense (relating to muscular strength or fever type).

^c *Паралогическим способом* (*paralogicheskim sposobom*): *paralogical* in the psychiatric usage of this period designates a specific mode of reasoning or conduct that proceeds according to an internal logic valid within the subject's own psychological system but disconnected from consensual reality or from the logical expectations of the social environment. The term derives from Greek *para-* (beside, beyond) + *logos* (reason), and its psychiatric usage descends from the German *paralogisch*, employed in descriptions of schizophrenic and paranoid thought. It does not mean simply *illogical* but rather *differently logical* — reasoning whose coordinates are displaced from consensual norms.

^d In the original, the Conclusions heading is set in spaced lettering — a standard Russian typographical convention for structural emphasis, functionally equivalent to small capitals or

bold display type in English-language printing of the period. The spaced rendering here approximates this device within the constraints of the present medium.

^e *Katatonische Bewegungsdrang*: catatonic motor drive — a German clinical term designating an irresistible urge toward motor discharge, associated in the literature of the period with catatonic symptomatology in schizophrenia. Sukhareva deploys it to describe the pathological intensification of the motor excitation pole of the schizoid's psychomotor bipolarity. The term is preserved untranslated, in accordance with Sukhareva's own editorial practice of embedding German terms of art within Russian prose without gloss — a convention consistent with the intensive German-language orientation of Soviet psychiatry in this period, in which the German clinical literature was read in the original as a matter of professional course.

^f *Закономерность* (*zakonomernost'*): rendered here as *regularity*, but the term carries a stronger connotation in the Soviet scientific lexicon of this period than mere statistical recurrence. Derived from *закон* (law), *закономерность* designates a *lawful regularity* — a pattern governed by an underlying necessity, not merely an observed frequency. In the natural-scientific and dialectical-materialist register of 1930s Soviet psychiatry, the term implies that the pattern in question is discoverable precisely because it is law-governed rather than contingent. The rendering *regularity* has been retained in the body text as the less loaded English equivalent appropriate to the clinical descriptive context of this passage; the stronger theoretical valence is recorded here.

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